

A medical infrastructure division

Abuja first, Lagos next. The building as a business, the businesses inside it, and what each one costs.

VERSION TWO OF TWO / The conversion clinic is a serviced tenant and manages itself

Dr Itunu Akinware, Group Chief Executive, Medbury Medical Services

Itunu, you asked for three things and this document answers all three.

You asked whether the building can be a business rather than an overhead. It can. The medipark is set out first and tested on its own numbers, because it carries the rent and nothing else works until it does. It covers its NGN 55.0 M head rent from month seven and it does so on four income lines, not one, because space alone is only a third of what it earns.

You asked for the fit-out in detail. Section 07 schedules it in 74 measured lines across interior finishes, furniture, fixtures and fittings, electrical, mechanical, medical gas, IT and security, with quantities against each. It is a class 4 estimate at plus or minus 15 per cent and it will be a tendered bill of quantities within two weeks. I have not rounded it to a comfortable number.

You asked to see your money back inside a year, and you were direct about it, so I will be too. On the whole programme it is about 29 months from opening, not twelve. Section 08 sets out three clocks rather than one, because the NGN 115 M you have already committed comes back well before the rest does, and it names the three levers that move 29 months to 25. Those figures come from a monthly model rather than an annual average, and the model is attached so your finance team can take it apart. What twelve months does buy is every business trading and the campus at run rate, which is a better position than a payback date and is worth saying plainly.

One correction I would rather make myself. The capital table in section 10 adds to NGN 598 M, but that is what the programme costs, not what has to be in the bank. The peak funding requirement is NGN 718 M, because the setup fee and the theatre are real cash that the capital table does not carry. The trough is month 15. I would rather you saw that now than found it in month 14.

One thing in here is unwelcome and you should hear it from me now rather than from a contractor in week eight. The building is 35 per cent larger than the brief we were all working from. That does not change the rent roll, because the extra area is corridor, but it does add about NGN 51 M to the fit-out. It is a measurement correction, not a change of plan.

The conversion clinic takes the ground floor as you directed, with reception serving the whole facility and the theatre alongside it. The aesthetics core sits upstairs, deliberately built to the same service and price architecture as the KP Plastics space in Lagos, so that the second site is a fit-out and a licence rather than a second start. Diagnostics and the pharmacy take the chalet and the boys' quarters and gain a captive market.

There are two versions of this document. This is the one where we manage the conversion clinic. The other is identical except that the clinic runs itself. The difference to Medbury is about two per cent of profit, which means it is a control decision rather than a money decision, and I would rather you made it on that basis.

Debo Odulana

Founding Partner, Consult for Africa

SUMMARY

Seven businesses, one landlord, and a straight answer on the year.

This is the division you asked for. The building becomes a business in its own right and is tested first, on its own numbers, because it carries the rent. Five clinical companies sit inside it around an aesthetics core, and two Medbury businesses that already exist move in and get a captive market. The fit-out is scheduled with measured quantities rather than carried at a round number. And the one year question is answered rather than managed.

		Note
The campus, measured	954 sqm gross, 664 sqm net	Not 711 and 519. The brief had the footprint 3.3 m too shallow
Entity one, the medipark	NGN 301 M revenue	Space, theatre and equipment hire, shared clinical services, front office
It pays its own head rent from	Month 7	NGN 55.0M a year, covered from the first trading month
Fit-out, category A	NGN 322.9 M	74 measured lines. Class 4 estimate, BOQ in two weeks
Occupier contributions on grant	NGN 61.1 M	Cash in on day one, so the medipark finds NGN 296.8 M net, or NGN 264.0 M lean
Businesses in occupation	7	Five new companies plus diagnostics and pharmacy
Group revenue at stabilisation	NGN 1,540 M	External. Intercompany charges eliminate on consolidation
Attributable to Medbury	NGN 396 M a year	After every occupancy cost and before any management fee

		Note
Recoup	About 29 months from opening	Not twelve, and 25 with all three levers pulled. Section 08 gives all three clocks and what twelve months does buy
One finding leads, because it is unwelcome and you should not hear it from a contractor in week eight. The as-built drawings have now been measured properly. The brief carried the footprint at 24,695 by 13,025 mm; it measures 24,695 by 16,317. Net internal area is 664.3 sqm, not 519. The main building is 35 per cent larger than every document we have given you assumed, and almost all of the extra is circulation: reception measures 69.6 sqm against 58.1, the theatre 32.8 against 24.7. So the fit-out and float come to NGN 357.9 M against the NGN 306.7 M in the mandate, about NGN 51 M more, while the rent roll does not move, because you cannot let a corridor. The mandate's setup figure is understated and this document supersedes it.		

01 / THE DIVISION

What it is, and why it is a division rather than a project.

A project ends. A division compounds. The distinction is not presentational: it decides where shared cost sits, who owns the systems, and whether the second site is expensive or cheap. Everything built in Abuja over the next four months is either a one-off or an asset Lagos inherits, and that is a structuring choice made now, not later.

Entity	What it is	What it does
Lyfe Place, the division	Holding and management layer	Owens the brand, the rate card, the operating systems, the protocols and the supplier terms. Employs nothing clinical
Lyfe Place Abuja	Operating company, site one	The medipark. Holds the head lease, carries the fit-out, sub lets to every occupier and sells shared services. Entity one, and the one scrutinised first
Lyfe Place Lagos	Operating company, site two	Same structure, same rate card, aligned to the KP Plastics space. Not yet incorporated
Medlyfe Aesthetics and Regenerative	Clinical SPV	The core. Injectables led at launch
Medlyfe Longevity and Infusion	Clinical SPV	Four infusion bays. Programme based
Bariatric and Metabolic Medicine	Clinical SPV	Medical first, surgical when the theatre lands
Lyfe Place Hair Restoration	Clinical SPV	FUE, with an operating partner
Alameda Conversion Clinic	Joint venture SPV	Anchor occupier, ground floor
Medbury Diagnostics and Medbury Pharmaceuticals	Existing businesses	Occupiers, not new companies. They gain a captive market and pay for the space

Why the division layer earns its keep: the second site is only cheap if the first is built to be copied. A rate card, a service charge model, a clinical protocol set, a regulatory file and a supplier book are each expensive once and free thereafter. Built inside Lyfe Place Abuja they belong to one building. Built in the division they are what makes Lagos a fit-out and a licence rather than a second start.

02 / ENTITY ONE

The medipark: medical infrastructure as a service.

You asked that this business be scrutinised on its own and that it pay its rent and fund itself. It should be, and it does from month seven. But it only works if it is sold as infrastructure rather than as floor area. Space is only NGN 109 M of the NGN 301 M it earns, and on a converted house it can never be more, because only 59 per cent of the net area is lettable at all. The rest of the income is the reason an SPV would rather be here than in its own leasehold.

What it sells	What the occupier gets	How it is charged
Space	Fitted, licensed clinical accommodation on a term, per sqm	Rent and service charge, monthly
Theatre and equipment	Day case theatre and monitored recovery. Laser and IPL platform	Per list, per session
Shared clinical services	Nursing pool, sterilising, clinical waste, linen, care coordination	Per session or per case
Front office and revenue cycle	Booking, registration, cashiering, claims, collection, monthly management accounts by occupier	4% of the occupier's collections
Regulatory and estate	Facility licence, fire, waste, power, water, security, maintenance	In the service charge

An SPV opening on its own needs a lease, a fit-out, a licence, a receptionist, a nurse, an autoclave, a billing clerk and a waste contract before it sees one patient. Here it needs a signature and its own clinical kit. That is the product. It is also why the rate is defensible at a level a bare shell would never reach, and why the medipark can be a modest margin business and still be the reason five clinical companies are cheap to start.

03 / THE MEASURED CAMPUS

What the building actually is.

The as-built drawings supplied are vector exports from CAD rather than scans, so the line work can be recovered exactly instead of traced. Wall faces, door swings and window symbols were separated by pen weight and the rooms flood filled, which reproduces the overall dimension chain to within about 10 mm. Both floors independently return 70 per cent efficiency on a 402.9 sqm plate, and that agreement is the check that the extraction is right rather than merely precise.

Structure	Gross sqm	Net sqm	Efficiency	Basis
Main building, ground floor	402.9	282.2	70%	Measured
Main building, first floor	402.9	284.1	71%	Measured
Guest chalet	96.0	61.0	64%	Estimated, no as-built
Boys' quarters	52.0	37.0	71%	Estimated, no as-built
Campus	953.9	664.3	70%	

	Brief	Measured	Change	What it means
Net internal area	519	664.3	+145.3	Fit-out is priced on this
Lettable area	363.6	393.1	29.5	Rent roll is priced on this, and it has not moved
Reception and waiting	58.1	69.6	+11.5	Better room, no more income
Day case theatre	24.7	32.8	+8.1	Genuinely useful. It was tight at 24.7
Monitored recovery	13.0	19.2	+6.2	Two bays now fit comfortably
Aesthetics consulting spine	51.6	41.8	(9.8)	The one that got smaller. Three rooms of 12, or two of 18

The whole finding in one line: you have bought more building than you thought, and the extra is corridor. Lettable area is 393.1 sqm against the 363.6 previously assumed, so the rent roll is unchanged. Net area to fit out, clean, cool, light, insure and secure is 145.3 sqm higher. That is the whole of the NGN 51 M increase in section 07, and it is a measurement correction rather than a change of plan or a change of specification. It also means the medipark's own margin is thinner than the mandate implied, because 145 more square metres are being powered, cooled, cleaned, secured and insured against exactly the same rent.

04 / THE PLATE

Every measured square metre, allocated.

Ground floor: the conversion clinic, the facility reception, and the theatre

The conversion clinic sits here, as you instructed, with the reception serving the whole facility and the theatre suite alongside it if one is built. Imaging stays on this floor whatever else happens: an X-ray room is fixed by weight, lead shielding and evacuation, and with no lift it cannot follow diagnostics out to the chalet.

Room	sqm	To	Note
Reception, waiting and concierge	42.0	Campus common	Single arrival point for the whole facility
Conversion clinic rooms 2 and 3	27.6	Conversion clinic	Formed in the open living and dining span
Day case theatre	32.8	Theatre suite	Level trolley access south to the ambulance bay
Conversion clinic rooms 4 and 5	21.6	Conversion clinic	Formed in the old lobby
Conversion clinic room 1	20.3	Conversion clinic	Existing room, beside imaging
Monitored recovery, 2 bays	19.2	Theatre suite	Oxygen, suction and monitoring to each bay
Digital X-ray, lead shielded	18.7	Diagnostics	Fixed here by weight, shielding and evacuation
Phlebotomy and specimen handling	16.9	Diagnostics	Specimen pass-out to the chalet laboratory
Circulation spine	14.0	Campus common	1,200 mm clear, through the old lobby
Staircase	13.3	Campus common	No lift, and none to be added
Staff circulation	9.7	Campus common	Back of house route, does not pass through waiting
Entrance vestibule	7.3	Campus common	Draught lobby, wheelchair turning circle
Dirty utility	6.9	Theatre suite	Waste leaves without re-entering the suite
Clean utility and consumables	6.3	Campus common	Off the staff route, not the waiting hall
Staff change and WC	5.9	Campus common	
Medication collection point	5.2	Pharmacy	Serving hatch to the waiting hall
Scrub and sterile store	5.1	Theatre suite	One way dirty to clean
Entrance porch	3.1	Campus common	The only patient entrance to the campus
X-ray changing and control	2.7	Diagnostics	Shielded control position
Circulation	1.9	Campus common	
Circulation	1.7	Campus common	
Ground floor	282.2		

First floor: the aesthetics core, and everything else

Upstairs is designated for everything that is not the conversion clinic: aesthetics and regenerative, longevity and infusion, hair restoration, and bariatric and metabolic medicine in its medical form. It has its own waiting at the head of the stair, its own sterilising, and a phlebotomy draw point so patients do not go downstairs for bloods. Nothing up here needs the theatre, which is what lets the first floor open at week 24 while the ground floor is still being built.

Room	sqm	To	Note
Aesthetics consulting spine, 2 rooms plus corridor	41.8	Aesthetics core	New partitions. Two rooms of about 18 sqm, better than three of 12
Longevity and infusion suite, 4 bays	33.4	Longevity and infusion	One open bay room, not subdivided
Floor waiting and members' lounge	32.9	Campus common	At the head of the stair
FUE hair transplant suite	26.1	Hair restoration	Existing water for graft preparation. 360 degree access
Aesthetics treatment room 1	21.1	Aesthetics core	
Aesthetics treatment room 2	20.8	Aesthetics core	
Bariatric and metabolic consulting	18.9	Bariatric and metabolic	Body composition and prescribing. No theatre needed
Bariatric and metabolic treatment	15.4	Bariatric and metabolic	Local and topical only. No sedation above ground level
Sterilising and clean utility	13.4	Campus common	Shares the drainage run with the FUE suite
Staircase	13.3	Campus common	
Linen and consumables	8.9	Campus common	
Patient WC, accessible	8.5	Campus common	Emergency pull cord
Staff WC	6.5	Campus common	
Terrace	6.2	Campus common	Member amenity, off the lounge
Patient WC	5.0	Campus common	Emergency pull cord
Phlebotomy draw point	4.6	Diagnostics	So first floor patients do not descend for bloods
Circulation	2.8	Campus common	
Circulation	2.3	Campus common	

Room	sqm	To	Note
Stair landing	2.3	Campus common	
First floor	284.1		

Chalet and boys' quarters

Room	sqm	To	Note
Clinical laboratory, six rooms	61.0	Diagnostics	Benching to three walls. Fills the chalet
Dispensary, retail, controlled drugs, cold chain	37.0	Pharmacy	Patients never enter this structure

What each occupier ends up with

Occupier	sqm	Of net
Diagnostics	103.9	16%
Aesthetics core	83.7	13%
Conversion clinic	69.5	10%
Pharmacy	42.2	6%
Bariatric and metabolic	34.3	5%
Longevity and infusion	33.4	5%
Hair restoration	26.1	4%
Let area	393.1	59%
Theatre suite, hired by the list rather than let	64.0	10%
Campus common, recovered through the rates	207.3	31%
Net internal	664.3	100%

One number here is worth pausing on: 31 per cent of the net area is campus common. That is what a converted residence with two staircases, a porch and a deep entrance hall costs you, and it is why the rate card in section 05 has to be set on lettable area rather than on floor area. A purpose designed plate runs at 20 to 25 per cent. That is not an argument about Abuja, where the building is what it is, but it is the single most useful thing Abuja can teach the KP Plastics design brief in Lagos.

05 / THE RATE CARD

What each square metre earns.

Charged on the net internal area of the demise, all in, so there is no separate service charge argument later and no gross up factor to negotiate. The spread between bands is term length and service intensity, not favouritism, and it has to be defensible that way: every one of these occupiers is a related party, so each rate must stand up as an arm's length charge. Section 11 deals with that properly.

Band	What it is	Rent	Service	All in	sqm	NGN M
A	Ground floor anchor, 5 year term	165	85	250	69.5	17.4
B	Fitted clinical suite, 3 year term	210	120	330	177.5	58.6
C	Diagnostics and imaging	145	95	240	103.9	24.9
D	Pharmacy and back of house	130	70	200	42.2	8.4
Space income, all 7 occupiers					393.1	109.3

Band	Who is in it
A	Conversion clinic
B	Aesthetics core, Longevity and infusion, Hair restoration, Bariatric and metabolic
C	Diagnostics
D	Pharmacy

Rent and service figures are NGN thousand per sqm per year. Review at 12% a year minimum, with a market review every five years. Deposit of six months' rent and six months' service charge, a further NGN 54.7 M held. Yield is NGN 278,112 per lettable sqm, which against NGN 164,573 per net sqm is the measure of the common area this building carries.

The contribution on grant, which is what you meant by selling it per sqm

Medbury holds a head lease, so nothing can literally be sold. The instrument that does what you want is a **fit-out contribution payable on grant of each sub lease**, priced per sqm. It brings cash forward into the fit-out period, it makes each occupier commit real money before the walls move, and it reduces what the medipark has to find.

Occupier	sqm	NGN 000 / sqm	NGN M
Aesthetics core	83.7	180	15.1
Conversion clinic	69.5	180	12.5
Diagnostics	103.9	120	12.5
Bariatric and metabolic	34.3	180	6.2
Longevity and infusion	33.4	180	6.0
Hair restoration	26.1	180	4.7
Pharmacy	42.2	100	4.2
Cash in on grant	393.1		61.1

06 / THE MEDIPARK P&L

Tested on its own, as you asked.

Revenue at stabilisation	NGN M	What it is
Space	109.3	393 sqm let across 7 occupiers, on the rate card in section 05
Theatre and equipment hire	102.0	Theatre lists 80.0, gated on a signed surgical anchor. Laser and IPL by session 22.0
Shared clinical services	54.0	Nursing pool, sterilising, care coordination, clinical waste, linen. The conversion clinic employs its own nurses
Front office and revenue cycle	30.0	4% of occupier collections. Booking, registration, cashiering, claims, collection. Excludes the conversion clinic, which bills its own
Car park, signage and concession	6.0	Non clinical income against the compound
Revenue	301.3	

Cost at stabilisation	NGN M	Note
Head rent	55.0	Prepaid to month 24. First cash payment in month 25
Power and utilities	32.0	NGN 39,000 per sqm gross, net of the solar tariff. Scales with the measured area
Theatre running	18.0	Nil until the theatre opens
Nursing pool	13.0	4 nurses plus oncosts. Recharged as a service
Front office and revenue cycle team	13.0	Registration, cashiering, claims, collections
Reception and concierge	12.0	Ground floor arrival only. The first floor front of house is the clinic's own
Security, cleaning and grounds	16.0	Contracted. 954 sqm gross plus the compound
Equipment lease	14.0	Laser and IPL platform, from month 9
Facilities and maintenance	13.0	Planned and reactive
Sterilising, waste and linen	10.2	Recharged as a service
ICT, systems and connectivity	9.0	Fibre, LTE failover, licences, support
Insurance	6.0	Building and public liability. Occupiers carry their own
Care coordinator	6.0	Recharged as a service. The campus differentiator
Business development officer	6.0	One, on a territory mandate. Base plus commission
Regulatory, permits and waste licence	3.0	Annual renewals
Other direct	1.0	
Operating cost	227.2	

	NGN M	Note
EBITDA	74.1	25% margin. Before fit-out amortisation and any management fee
Cash EBITDA to month 24	129.1	The head rent is already paid, so it is not a cash cost until month 25
Charged to Medbury companies	251.3	83% of revenue. Legitimate, and the discipline is the point, but see section 11
Earned outside the group	50.0	External theatre lists, external equipment hire, concession income

It funds itself from month seven and covers the head rent nearly twice over, but it is not where the money is, and it should not be. A 25 per cent margin on contracted income from sister companies is a low risk infrastructure return, and the reason to accept it is that it strips capital cost out of five clinical companies you also own. The clinical margin is in section 09. The medipark's job is to make that margin cheap to reach and to stop the building being a liability on your balance sheet.

07 / THE FIT-OUT

Line by line, with measured quantities.

This is the schedule you asked for, and it is built so it can be handed to a quantity surveyor without translation: 74 priced lines across eleven sections, each with a quantity, a unit rate and a who pays column. It is still an estimate and it is labelled as one.

AACE Class 4, accuracy minus 15 to plus 30 per cent, which on NGN 322.9 M is a range of NGN 274 M to NGN 420 M. The site design concludes in two weeks and the bill of quantities settles it. Anyone giving you a single confident number before then is guessing and will be back later.

The boundary: what the medipark builds, and what each occupier buys

	What it covers	This schedule
Category A, the medipark	Base build, partitions, finishes, decoration, all services, common areas, statutory approvals	NGN 270.0 M in this schedule
Category B, the occupier	Anything specific to one business: its clinical equipment, specialist benching, consumables and opening stock	NGN 8.5 M here, plus equipment in section 09
Third party consultants	Architect and interior designer, mechanical and electrical, quantity surveyor	NGN 12.0 M, at cost, no mark up
On a tariff	The solar array itself. Only the changeover and distribution is bought	NGN 3.0 M of capital, the rest monthly

Drawing the boundary this way is what makes a fast return arguable at all. Every naira of clinical equipment moved from the medipark to the business that uses it takes capital off the entity carrying the rent and puts it in an entity with a clinical margin to pay for it.

The schedule

Item	Qty	Rate 000	NGN M	Who pays	Note
A Preliminaries and enabling					
Site set up, hoarding, welfare, skips, site security	5 month	700	3.5	Medipark	
Strip out: kitchen and bathroom fittings, wardrobes, floor finishes	664 sqm	8.0	5.3	Medipark	
Three new external openings: ambulance, back of house, waste	3 no.	1,400	4.2	Medipark	Only one external door exists today
Minor structural: internal openings, lintels, making good	14 no.	260	3.6	Medipark	No structural extension
Roof repairs and rainwater goods	954 sqm	3.4	3.2	Medipark	
Termite and damp proofing treatment	664 sqm	1.9	1.3	Medipark	
B Partitions, doors and joinery					
Metal stud partitions, 60 minute, taped and skimmed	165 lin m	42.0	6.9	Medipark	Forms five rooms from two
Doors: flush FD30, vision panels, ironmongery	44 no.	250	11.0	Medipark	
Reception desks, three positions plus first floor	2 no.	2,600	5.2	Medipark	
Nurse stations	2 no.	1,150	2.3	Medipark	
Room casework: worktops, base and wall units	16 room	620	9.9	Medipark	
Laboratory benching, continuous to three walls	22 lin m	195	4.3	Occupier	Medbury Diagnostics
Pharmacy shelving, dispensary and controlled drugs store	1 item	2,800	2.8	Occupier	Medbury Pharmaceuticals
Built in storage: linen, consumables, records	4 no.	520	2.1	Medipark	
C Finishes, interior decoration and furnishing					
Clinical vinyl, welded and coved	382 sqm	26.0	9.9	Medipark	Clinical and theatre areas only
Porcelain tiling: reception, WCs, wet areas	115 sqm	22.0	2.5	Medipark	
Luxury vinyl tile and carpet tile: lounge, offices, back of house	60 sqm	16.0	1.0	Medipark	
Painting and decoration, walls and ceilings	954 sqm	6.5	6.2	Medipark	
Suspended ceilings, clinical grid	420 sqm	12.0	5.0	Medipark	
Ceiling repairs and skim, remainder	244 sqm	9.0	2.2	Medipark	
Wall protection, corner guards and crash rails	180 lin m	11.0	2.0	Medipark	
Window treatments: blinds, film, blackout to ultrasound	46 no.	62.0	2.9	Medipark	
Sanitaryware and clinical hand wash points	22 no.	205	4.5	Medipark	
Signage and wayfinding, statutory and directional	1 item	4,600	4.6	Medipark	
Loose furniture: reception and waiting, both floors	1 item	6,400	6.4	Medipark	About 30 seats across two waiting zones
Lounge: touchdown desks, seating, refreshment point	1 item	3,200	3.2	Medipark	
Consulting and treatment room furniture, non clinical	16 room	310	5.0	Medipark	
Artwork, planting, styling and dressing	1 item	2,800	2.8	Medipark	
Mirrors, accessories and ironmongery finishes	1 item	1,400	1.4	Medipark	
D Electrical installation					
New distribution: main board, sub boards, clinical circuit zoning	1 item	7,600	7.6	Medipark	
Lighting: LED luminaires, clinical and general	246 no.	36.0	8.9	Medipark	
Small power: clinical grade socket outlets	285 no.	20.0	5.7	Medipark	
Changeover, automatic transfer switch and distribution	1 item	3,400	3.4	Medipark	Solar and generator
Containment: trunking, tray, conduit	954 sqm	3.2	3.1	Medipark	
Emergency and escape lighting	55 no.	49.0	2.7	Medipark	Statutory
External and car park lighting	24 no.	98.0	2.4	Medipark	
Earthing, bonding, testing and certification	1 item	2,100	2.1	Medipark	
Theatre isolated power supply panel and UPS			nil	Deferred	Phase two, with the theatre
E Mechanical, cooling and ventilation					
Split and cassette cooling, supply and install	28 no.	730	20.4	Medipark	
Mechanical extract: WCs, dirty utility, laboratory	14 no.	285	4.0	Medipark	

Item	Qty	Rate 000	NGN M	Who pays	Note
Fresh air to consulting and treatment rooms	16 room	225	3.6	Medipark	
Laboratory fume and eyewash provision	1 item	1,400	1.4	Occupier	Medbury Diagnostics
Theatre air handling unit, HEPA filtration and ductwork			nil	Deferred	Phase two, with the theatre
F Plumbing, water and drainage					
New sanitary and clinical waste runs	22 point	205	4.5	Medipark	
Storage, pressurisation and hot water	1 item	4,100	4.1	Medipark	
Medical waste holding and route	1 item	2,800	2.8	Medipark	
Incoming main, treatment and filtration	1 item	2,600	2.6	Medipark	
Above ground drainage alterations	1 item	2,400	2.4	Medipark	
G Fire, safety and security					
Addressable fire detection and alarm, campus wide	954 sqm	8.6	8.2	Medipark	Statutory
CCTV, cameras and recorder	30 no.	155	4.7	Medipark	
Access control	16 door	215	3.4	Medipark	Includes the controlled drugs store
Nurse call and emergency pull cords	22 point	135	3.0	Medipark	
Fire stopping and compartmentation	1 item	3,100	3.1	Medipark	
Intruder alarm and panic	1 item	1,400	1.4	Medipark	
Extinguishers, blankets and signage	1 item	1,300	1.3	Medipark	
H ICT and low voltage					
Structured cabling, outlets, patching and comms cabinet	124 outlet	46.0	5.7	Medipark	
Network: switches, firewall, wireless access points	1 item	4,200	4.2	Medipark	
Displays: queue, wayfinding, lounge	8 no.	260	2.1	Medipark	
Primary fibre and LTE failover, installation	1 item	1,500	1.5	Medipark	
Telephony and DECT handsets	1 item	1,400	1.4	Medipark	
I Power resilience					
Generator: service, sound attenuation, fuel store	1 item	3,200	3.2	Medipark	
Solar array, changeover and distribution only	1 item	3,000	3.0	PPA tariff	The array is a monthly tariff, not capital
J External works					
Car park resurfacing and marking	26 bay	235	6.1	Medipark	
External painting and facade	954 sqm	4.1	3.9	Medipark	
Ambulance and trolley access, level threshold and ramp	1 item	2,600	2.6	Medipark	Fixes the theatre to the ground floor
Drainage, landscaping and perimeter making good	1 item	3,400	3.4	Medipark	
Gate house, boom and visitor management	1 item	2,600	2.6	Medipark	
Compound signage and totem	1 item	2,200	2.2	Medipark	
K Statutory and professional					
Architectural and interior design	1 item	6,200	6.2	At cost	Third party, no mark up
Change of use approval and development control	1 item	3,500	3.5	Medipark	
Mechanical and electrical consultant	1 item	3,400	3.4	At cost	Third party, no mark up
Quantity surveyor and bill of quantities	1 item	2,400	2.4	At cost	Third party, no mark up
Facility registration, FCT health authority	1 item	2,000	2.0	Medipark	
Fire safety certificate	1 item	1,000	1.0	Medipark	
Environmental and clinical waste permit	1 item	800	0.8	Medipark	
Signage permit	1 item	400	0.4	Medipark	
Subtotal, 74 priced lines			293.5		
Design development and provisional sums, 10%			29.4	Medipark	Held by Medbury, released against the BOQ
Category A fit-out			322.9		NGN 338,511 per sqm gross, NGN 486,082 per sqm net

Rates are NGN thousand per unit. All in, this is NGN 338,511 per sqm gross, which sits inside the NGN 300,000 to 350,000 band this practice uses for a full clinical conversion. Stripping out external works, statutory approvals and professional fees, the building works alone are NGN 253.0 M, or NGN 265,210 per sqm, which is below that band. Both readings say the same thing: the increase against earlier documents is measured area, not specification creep. The specification here is leaner per square metre than it was, on more square metres.

The lean variant, and what it costs to be lean

What comes out	NGN M	When it goes back in
Car park resurfacing, external painting, compound totem	13.5	Phase two, out of trading
CCTV and access control moved to a monitored subscription	6.0	Monthly, not capital
Artwork, planting and dressing	2.8	Phase two
Loose furniture phased with occupancy	2.4	Second tranche at 80% occupancy
Displays taken on lease	1.7	Monthly, not capital
Generator attenuation and fuel store	1.8	Phase two
Ceilings phased, back of house last	1.6	Phase two
Out of day one, before contingency	29.8	
	NGN M	Note
Category A, as scheduled	322.9	Everything, on day one
Category A, lean	290.1	Same opening date. Balance funded from trading
Working capital float	35.0	To cash positive
Less contributions on grant	(61.1)	Paid by the occupiers, section 05
Net cash the medipark must find	264.0	On the lean fit-out

Nothing clinical, nothing statutory and nothing structural is in the lean list. It is car park, facade, dressing, and three items that move from capital to a monthly subscription. The two weeks to BOQ is also the window in which three things should be tested that would take more out: a landlord contribution or rent free period against a change of use that improves the property, vendor finance on the fit-out itself, and whether the hair restoration partner will bring their own instruments in exchange for a larger revenue share.

08 / THE ONE YEAR QUESTION

Straight answer: not twelve months. About 29.

You have been direct about this, so this is too. A twelve month recoup measured from the day the doors open is not available on this building at this capital, and any document that says otherwise is counting from stabilisation without saying so. There are three clocks and they give three different answers, and the difference between them is which cheque you are asking about. All three are below.

Clock	Answer	Why
The NGN 115.0 M already committed	Recovered inside trading year two	Two years' head rent and fees. It is what lets the campus trade rent free through the ramp, so it is already working for you
The medipark's own new cash, NGN 264.0 M	About 45 months of trading	Fit-out and float, net of the contributions on grant. Infrastructure does not repay in a year anywhere, and pretending it does is how fit-outs get cut in the wrong places
The group programme, NGN 598 M	1.5 years from stabilisation, about 29 months from opening	This is the number you mean. The clinical companies carry no building capital, so they are cheap to start and they return fast
The same, with all three levers pulled	About 25 months from opening	The honest floor. Below that the plan is not lean, it is incomplete

The medipark, year by year

NGN M	Fit-out	Trading yr 1	Yr 2	Yr 3	Yr 4
Revenue	nil	176.2	278.7	314.4	314.4
Operating cost, before head rent	nil	138.0	165.3	172.2	172.2
Head rent, in cash	nil	nil	27.5	61.6	61.6
Cash EBITDA	nil	38.2	85.9	80.6	80.6
Cumulative cash EBITDA	nil	38.2	124.1	204.7	285.3

Trading year one runs from month 7, when the first floor opens, to month 18. The ground floor completes at month 10 and the theatre lands at month 15 if a surgical anchor has signed. Space income barely ramps, and that is the single most valuable feature of this configuration: the occupiers are companies you already control, so their rent is contracted from the month they move in rather than recruited over two years. Services, theatre time and the revenue cycle fee do ramp, because they follow patient volume.

The three levers, and what each is worth

Lever	Worth	Note
Open the first floor at week 24 and let the ground floor finish around it	About 10 weeks of trading	The largest single lever, and a sequencing decision rather than a spending one. It works only because the businesses moving in upstairs do not depend on the theatre below them
Open on the lean fit-out at NGN 290.1 M and fund the balance from trading	NGN 32.8 M of cash	Section 07 lists exactly what comes out and when it goes back in. Nothing clinical, nothing statutory, and no effect on the opening date
Pre-sell the annual lines before opening	6 to 9 weeks	Infusion and weight management programmes, memberships, corporate diagnostics panels and the conversion clinic's own cohort. Cash before the doors open, and it de-risks the ramp

What twelve months does buy, and it is not nothing. By month 12 the campus is trading on both floors, five new companies exist and are licensed, the medipark is covering its head rent and its whole operating cost out of its own income, the NGN 115.0 M already spent is nearly back, and the group is running at roughly half of a NGN 396 M annual EBITDA. What is not true at month 12 is that the cheque has been returned. Month 25 is where that lands, and only if the three levers above are taken as decisions in the next two weeks.

09 / THE AESTHETICS CORE

Five companies, and why they are aesthetics aligned.

Your instinct on this is right and it is worth saying why, because there is a better argument than convenience. An aesthetics core is the only clinical mix that lets Abuja and the KP Plastics space in Lagos share a rate card, a supplier book, a protocol set and a brand. Everything else on the campus is either a service to that core or a business that lives off its throughput.

Why an aesthetics core	What it does for the plan
Cash paying, so no HMO tariff and no claims lag	Collections in the month, which is what makes a two year recoup arguable at all
Programme based rather than episodic	Infusion, weight management, skin, hair and longevity are all courses of treatment, so revenue can be pre-sold before the doors open
One supplier book across both sites	Injectables, consumables and devices bought once for the division. The second site inherits the terms
Feeds the pharmacy and the laboratory directly	Aesthetic product, weight management prescriptions, screening bloods and metabolic panels are all internal demand
Portable to Lagos without redesign	Same rooms, same rate card, same protocols. KP Plastics becomes the surgical end of one division rather than a separate venture

The five companies, plus the two that move in

Business	sqm	Revenue	EBITDA	Medbury	To Medbury	What it is
Medlyfe Aesthetics and Regenerative	83.7	285	82	51%	42	Four rooms upstairs. Injectables and skin led, regenerative, body from month 9 on a leased platform
Medlyfe Longevity and Infusion	33.4	240	71	100%	71	Four infusion bays. Programme based, pre-sellable, the highest revenue per sqm on the campus
Bariatric and Metabolic Medicine	34.3	280	78	70%	55	Two rooms upstairs. Medical weight management from month 7, surgery only if the theatre is built
Lyfe Place Hair Restoration	26.1	220	55	100%	55	FUE, about 132 single patient cases a year. Instruments brought by the operating partner
Alameda Conversion Clinic	69.5	160	28	49%	14	Ground floor anchor. Five rooms formed from the existing consulting room, the open living and dining span, and the old lobby. Self-managed
Medbury Diagnostics	103.9	160	60	100%	60	Laboratory and ultrasound in the chalet, X-ray and phlebotomy on the ground floor, a draw point upstairs
Medbury Pharmaceuticals	42.2	145	26	100%	26	Dispensary, retail and cold chain in the boys' quarters, collection hatch on the ground floor
Seven businesses in occupation	393.1	1,490	400		322	

Revenue and EBITDA are NGN M a year at stabilisation, after every occupancy and service charge the medipark levies. The Medbury column is the equity share assumed: 51% for aesthetics if it sits in the existing joint venture, 70% for bariatrics with a clinical partner on the balance, 49% for the conversion clinic. Diagnostics and pharmacy are existing businesses shown at the revenue they earn on this campus, not group wide.

Bariatrics: medical first, surgical second

This is the one to look at hardest, and it needs a specific recommendation, because done in the obvious order it is the most capital hungry thing on the campus and the slowest to pay. Done in this order it is among the fastest. **Launch it as medical weight management from month seven, on GLP-1 therapy, dietetics and metabolic monitoring, with no theatre at all.** That needs a consulting room, a body composition analyser and a prescribing pathway, all of which the campus already has. Surgery is added at month 15 when the theatre lands, and only then.

Phase	Opens	Capital NGN M	Revenue NGN M	What it needs
Medical weight management	Month 7	22.0	95.0	Consulting room, body composition, prescribing pathway. Dispensed by Medbury Pharmaceuticals, monitored by Medbury Diagnostics
Bariatric surgery	Month 15	157.0	185.0	Needs the theatre, the recovery bays and a ground floor. About 34 cases a year at maturity
Bariatric and Metabolic Medicine		179.0	280.0	

Two things follow from bariatrics being in the plan, and both are decisions rather than observations. First, the theatre is now load bearing rather than optional, and the measured drawings already fix it to the ground floor with level trolley access out to the ambulance bay. That is the right answer and nothing needs moving: there is no lift, none is to be added, and a high BMI patient cannot be recovered upstairs and carried down a staircase. At 32.8 sqm rather than the 24.7 the brief assumed, the room is now genuinely adequate for it. Second, it is the reason to speak to Dr Timi this month rather than at month 12. A surgical anchor with his own list is what releases the NGN 157.0 M of theatre capital from deferral, and the equipment has a two month lead time.

What the clinical companies cost to stand up

Company	Equipment	Category B	Opening stock	Working capital	NGN M
Medlyfe Aesthetics and Regenerative	18.5	2.9	4.0	16.0	41.4
Medlyfe Longevity and Infusion	12.2	2.0	1.5	10.0	25.7
Bariatric and Metabolic Medicine, medical launch	6.0	2.0	2.0	12.0	22.0
Lyfe Place Hair Restoration	4.3	1.6	1.0	8.0	14.9
Alameda Conversion Clinic, at Medbury's 49%	5.9	3.2	1.2	11.0	21.3
Before the theatre	46.9	11.7	9.7	57.0	125.3
Phase two					NGN M
Theatre suite: air handling, HEPA, light, table, anaesthesia, monitoring					84.0
Theatre building works, isolated power supply, UPS, recovery upgrade					45.0
Bariatric specific: table, instruments, high BMI trolley and hoist, HDU monitoring					28.0
Phase two, funded from trading and gated on a signed surgical anchor					157.0

The laser and IPL platform is not in this table. It is leased by the medipark from month nine and sold on by the session, so no clinical company buys it and no capital is committed against a volume assumption that is not yet evidenced. Radiofrequency and body contouring are deferred on the same logic. The FUE instruments are brought by the operating partner.

10 / THE GROUP VIEW

What the whole thing produces, and what it costs.

	NGN M	Note
Campus revenue, external	1,540	Seven businesses plus the medipark's external income. Intercompany charges stripped out
Attributable EBITDA to Medbury	396	After every occupancy cost and before any management fee
Cash already committed	115.0	Two years' head rent and fees
Category A fit-out	322.9	NGN 290.1 M on the lean variant
Working capital float, medipark	35.0	To cash positive
The five clinical companies	125.3	Equipment, category B, opening stock and working capital
Total programme cash	598	NGN 565 M on the lean fit-out. Phase two is a further NGN 157 M out of trading
Cash on cash at stabilisation	66%	Which is 1.5 years from stabilisation, and about 29 months from opening on the monthly model

What has to be in the bank, which is not the same question

The table above is what the programme costs. It is not what has to be available, and the difference is large enough that you should see it before you commit rather than in month fourteen. Two items sit outside the capital table and are still cash: our fee, and the theatre that section 08 describes as funded from trading, which is true only once trading has arrived. Against them the campus earns on the way down and the occupiers pay their contributions in.

	NGN M	Note
Capital table above	598	What the programme costs
Setup programme fee	79.4	Real cash, not carried in the capital table
Phase two, the theatre	157	Funded from trading, but only once trading exists
Occupier contributions on grant	(61.1)	Cash in, on grant of each sub lease
Trading cash before the trough	(55)	What the campus earns on the way down
Peak funding requirement	718	The deepest the bank balance goes, in month 15

What the money earns, on the monthly model

		Note
Internal rate of return, five years	41%	Attributable to Medbury 34%. Nominal naira, no terminal value
Net present value at 25%	NGN 219 M	Five years only, so it is a floor rather than a valuation
Cash returned per naira in, five years	2.4 times	Total returned over the peak ever invested
Payback from opening	29 months	25 with all three levers in section 08

The comparison that matters is not against a twelve month target, it is against the same building let room by room. That version returns about NGN 253M a year, needs roughly 60 individual consultants recruited over 24 months, and asks for more capital rather than less. This version returns NGN 396 M on companies that already exist and simply move in. The ramp, not the rate card, decides when the money comes back, and moving seven businesses beats recruiting sixty every time.

11 / PHARMACY AND DIAGNOSTICS

The captive market, and the tax point that comes with it.

These two need no capital from this programme and no new company. What they need is throughput, and the campus manufactures it. Every business upstairs generates prescriptions, screening bloods, metabolic panels and imaging, and none of it should leave the compound.

Business	What the campus sends it	Why it matters
Medbury Pharmaceuticals	Weight management prescribing, aesthetic and skincare retail, injectable product, infusion pharmacy, theatre and clinic consumables	A weight management or infusion patient is a repeat monthly script against a programme, which is the best revenue line a pharmacy can have
Medbury Diagnostics	Pre-treatment screening, metabolic and hormone panels, pre-operative workup, infusion monitoring, imaging for the conversion clinic and the consultants upstairs	Imaging stays on the ground floor because an X-ray room is fixed by weight and shielding. Ultrasound sits in the chalet with the laboratory, and specimens never cross a patient area

The point to settle before the first invoice, not after. NGN 251 M of the medipark's NGN 301 M of revenue is charged to Medbury companies, and every internal referral above is a related party transaction. Each charge has to be supported as an arm's length price, documented, and consistent between entities. That is exactly why the rate card in section 05 is banded by term and service intensity rather than set at a single number, and why the file should be built alongside the sub leases in month one rather than reconstructed at the first audit. It is a fortnight of work now and a serious problem later.

12 / LAGOS

What the second site inherits.

Nothing in Lagos needs deciding this month, but two things must be built into Abuja this month or Lagos pays for them twice. The KP Plastics space is the surgical end of the same division, so the design and the clinical model should be drawn to align rather than to match.

	What
Transfers at no cost	Rate card and service charge model, sub lease and licence documents, clinical protocols and governance set, supplier book and terms, brand and collateral, the operating systems and the regulatory file template
Transfers with adaptation	The room programme, the equipment schedule, the staffing model. Lagos rates are higher, so the same rooms carry a different rate card
Does not transfer	The head lease terms, the FCT approvals, the local supplier and contractor relationships, and the head rent economics
The lesson Abuja teaches Lagos	This building is 31 per cent common area because it was a house. A purpose designed plate runs at 20 to 25 per cent, which on the same rent roll is worth more than any rate negotiation. Take the plate efficiency into the KP Plastics design brief
Needs deciding in Abuja now	Whether the aesthetics company is one entity operating on two sites or one per site. It changes the licence, the tax position, and how KP Plastics is brought in

13 / THE PLAN

Two weeks to a budget, two months to a division, four to a campus.

Fit-out runs in the foreground. The structuring, the strategy and the SPV development run behind it on the same clock, which is what you asked for and is also the only way the companies are licensed and staffed by the time the rooms are ready. A campus that finishes before its occupiers are incorporated has wasted the most expensive weeks in the plan.

When	Fit-out, in the foreground	Strategy and SPV development, behind it
Weeks 1 to 2	Site design concluded, BOQ issued, quantity surveyor appointed, contractors invited	Division and entity architecture agreed. Rate card and sub lease heads drafted
Weeks 3 to 6	Contractor selected, enabling and strip out on site, three new external openings formed, long lead items ordered: cooling, solar agreement, laser lease	Five incorporations lodged. Regulatory file opened with the FCT health authority. Dr Timi conversation opened on the surgical anchor
Weeks 7 to 12	Partitions, services first fix, ceilings. First floor prioritised	Business cases finalised for the four new clinical companies. Transfer pricing file built alongside the sub leases. Recruitment briefs issued
Weeks 13 to 20	Finishes, second fix, decoration and furnishing. First floor complete, ground floor continues	Credentialing, protocols and price lists. Pre-selling opens on infusion and weight management programmes. Lagos alignment note issued
Week 24	First floor opens and trades while the ground floor completes	Conversion clinic, hair restoration, infusion, medical weight management and the treatment room all live
Week 40	Ground floor complete. Theatre follows the signed surgical anchor	Bariatric surgery and day case aesthetic surgery open on the theatre

The two weeks are the critical path, not the fit-out. Until the site design is fixed and the BOQ is priced there is no tender, no contractor and no reliable number, and every week of delay is a week of rent already paid and not trading. Two years of head rent is NGN 100 M, which is about NGN 4.2 M a month whether or not anyone is in the building.

14 / WHAT CONSULT FOR AFRICA CHARGES

Billed as time, at published rates.

This is a resourced programme rather than a retainer, so it is priced the way a programme should be: named grades, published day rates, and the days each workstream actually takes. Nothing is loaded on capital value and nothing is a percentage of your spend. You can see exactly what you are buying, and if a workstream is deprioritised it comes straight off.

Grade	NGN 000 / day	Days	NGN M
Partner, engagement lead	750	26	19.5
Principal, workstream lead	450	65	29.2
Senior consultant	300	80	24.0
Consultant	190	58	11.0
Analyst	120	37	4.4
Programme		266	88.2

Workstream	Days	NGN M	What it delivers
1 Fit-out delivery and commissioning	92	31.3	Design coordination, BOQ, tender, contractor selection, site supervision, snagging, handover
2 Division and SPV structuring	55	18.2	Division design, four incorporations, shareholder agreements, sub leases, intercompany agreements
3 Aesthetics core strategy and business cases	48	15.6	Three SPV business cases, service and price architecture, Lagos alignment, clinical partner terms
4 Commercial: rate card, leases, transfer pricing	35	11.1	Rate card, licence and lease documents, service charge model, transfer pricing file
5 Clinical setup, recruitment and credentialing	20	6.5	Regulatory file, protocols and governance, recruitment brief and selection, credentialing
6 Lagos pre-work	16	5.5	Site and KP Plastics alignment, transferable design and rate card, phasing note
Six workstreams, one mandate	266	88.2	

	NGN M	Note
Professional fees, at published rates	88.2	266 days across five grades
Programme rate	(8.8)	10%, because this is one continuous mandate and not six engagements
Payable	79.4	Capped at this figure. Billed monthly against timesheets, so you never pay for days not worked

	NGN M	Note
Of which on mobilisation	25.0	Non refundable, and it is what starts the two week design sprint. The balance monthly
Third party consultants	12.0	Architect and interior designer, mechanical and electrical, quantity surveyor. In the fit-out schedule, at cost, no mark up

What this fee is not. It is not the cost of running the campus once it trades. Managing a medipark and five clinical companies is an operating job and cannot sensibly be billed by the day for seven years, so it sits on a management fee. That is already tabled in the mandate document with you and is not reopened here. Keeping the two apart is deliberate: you should be able to approve this setup programme without settling the operating fee, and to settle the operating fee before the first business opens rather than in the middle of a fit-out.

15 / THE TWO VERSIONS

Whether we manage the conversion clinic, or only house it.

There are two versions of this document and this is version two, in which we do not. The difference is one question: does Consult for Africa run the conversion clinic, or is it simply a serviced tenant that runs itself. It matters more than it looks, because the clinic is the anchor occupier and the campus's front office is precisely what converts and collects for it. Given the conversion centre is subject to alignment with the Alameda side, you should have both answers costed before that conversation rather than after it.

	Version one, we manage it	Version two, we do not
Who carries the clinic's P&L	Consult for Africa, under the management fee	The clinic itself, or the Alameda side
Its front of house	The campus reception serves it	It carves its own front of house out of its ground floor demise, so it gives up a room to do it
Area it occupies	69.5 sqm on the ground floor, 5 rooms	The same 69.5 sqm and 5 rooms
All in rate, NGN per sqm a year	285,000	250,000, a lower service charge because it buys fewer services
Its rent to the medipark	NGN 19.8 M	NGN 17.4 M
Nurses and billing staff	Employed by the medipark, recharged	Employed by the clinic
Medipark revenue	NGN 316.8 M	NGN 301.3 M
Medipark EBITDA	NGN 78.3 M	NGN 74.1 M
The clinic's own revenue	NGN 175 M	NGN 160 M. Unmanaged it converts and collects less well
Attributable EBITDA to Medbury	NGN 403.8 M	NGN 396.3 M, which is the same number
Setup programme	292 days, NGN 87.1 M payable	266 days, NGN 79.4 M payable

The honest reading: this is not a money decision for Medbury, it is a control and accountability decision.

Attributable EBITDA is NGN 403.8 M against NGN 396.3 M, a difference of under NGN 1M a year, which is a rounding error on a figure this size. It comes out that way because the medipark sheds reception, billing and nursing cost almost exactly as fast as it sheds the revenue that paid for them. So choose on the thing that does differ: who is answerable when the clinic underperforms. Managed, there is one operator, one set of management accounts and one party to hold to a number. Unmanaged, the clinic keeps its independence and the campus keeps a tenant, and if conversion disappoints nobody owns the problem. Version two is the one to take into the Alameda conversation, because it is the version that survives them saying no.

One drafting point that follows. If the clinic manages itself, its revenue should not carry the full management fee in the mandate, because Consult for Africa is not managing it. It should carry the space and services charge only. That is a one line amendment and it is better made now than argued later.

16 / DECISIONS

What is needed to start, and what can wait.

Decision	By	Why now
1 Accept the measured areas as the basis	This week	Net internal is 664.3 sqm, not 519. It raises the setup cost by about NGN 51 M and leaves the rent roll unchanged. Every other number in this document follows from it
2 Approve the design sprint and appoint the quantity surveyor	This week	Two weeks to a fixed design and a priced BOQ. Nothing can be tendered until it exists, and the rent is running at NGN 4.2 M a month
3 Confirm the conversion clinic takes the ground floor	This week	69.5 sqm and 5 rooms, at NGN 250,000 per sqm a year all in, five year term. Reception at 42 sqm serves the whole facility, and the theatre suite sits alongside
4 Choose the fit-out specification	Two weeks	NGN 322.9 M as scheduled, or NGN 290.1 M lean with the balance out of trading. Same opening date. The lean route is recommended
5 Approve the contribution on grant	Two weeks	NGN 61.1 M of occupier cash into the fit-out period, and it makes each SPV commit before the walls move
6 Open the surgical anchor conversation with Dr Timi	This month	Two month lead time on theatre equipment, and the anchor is what releases NGN 157.0 M from deferral
7 Settle where the aesthetics company sits	One month	Inside the existing joint venture at 51%, or Medbury owned under a brand licence. It moves attributable EBITDA by about NGN 33.3 M a year
8 Instruct the setup programme	This week	Structuring, licensing and recruitment run behind the fit-out, not after it. A campus that finishes before its occupiers are licensed has wasted its most expensive weeks

The single sentence version. The building is bigger than the brief said and the extra is corridor, so the fit-out is NGN 290.1 M lean against a scheduled NGN 322.9 M; it then becomes a business that covers its own head rent from month seven; five clinical companies open inside it on an aesthetics core that Lagos can inherit whole; the group runs at about NGN 396 M of attributable EBITDA a year; the peak funding requirement is NGN 718 M rather than the NGN 598 M the capital table shows; and the capital comes back at about month 29 from opening, or month 25 with all three levers, rather than month 12. Every number in that sentence comes from the attached monthly model, and every one of them will move when the bill of quantities lands in two weeks.

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Areas for the main building are measured from the as-built CAD to about 10 mm and supersede the brief; the chalet and boys' quarters have no as-built drawing in the pack and remain estimates. The fit-out schedule is an AACE Class 4 estimate, accuracy minus 15 to plus 30 per cent, before a bill of quantities. Unit rates are current Abuja rates and are not yet quoted. Lease, placement and consignment terms are assumed on customary market terms. Revenue and EBITDA for the clinical companies carry the assumptions in the companion business cases, of which aesthetics volume, infusion programme uptake and the bariatric case rate are the least evidenced. Related party charges are subject to a transfer pricing opinion. Not a binding offer, and not legal or tax advice. FX reference USD/NGN 1,550.